Mass per abdomen

PRESENTER: DR SAHANA M
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GENERAL SURGERY DEPARTMENT
KVGMCH
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Personal details:

- Name : Mr. ABC
- Age : 70 years
- Gender : Male
- Occupation : Labourer
- Address : Periyapatna, Mysore
- Date of admission : 27th July 2020
- Date of examination : 30th July 2020

Chief Complaints:

Abdominal pain since 6 months

Vomiting since 1 month

History of present illness:

- Patient complains of **abdominal pain** since 6 months- in the upper half of abdomen, sudden in onset, initially it was dull aching in nature (with a pain free period), non-radiating, aggravated by food ingestion and relieved by antacids. It didn't affect his daily activities and there was no episode of nausea or vomiting, water brash or heartburn sensation then.
- But since the past one month it has gradually increased in severity and is continuously present.

- Patient also complains of **vomiting** since 1month- sudden onset, occurs immediately after meals (within few mins), non-projectile, non-bilious, not blood stained, contained water and food particles.

- Patient gives history of passing **dark tarry stools** since 15days, that sticks to the toilet seat and there is reduced frequency of bowel movements, once in 3-4 days. There is no history of consumption of any iron supplements.

- History of **loss of appetite** present since 1 month
- History of **weight loss** is present (patient was weighing 52kgs one month ago and now he weighs 45kgs. He has lost 7kgs weight in a duration of one month)
- History of generalized weakness and easy fatiguability present since 1 month which has affected his daily activities
- There is no history of yellowish discoloration of the urine, eyes and skin.
- No history of breathlessness/hemoptysis
- No history of bony pains
- Not a known case of Diabetes Mellitus, Hypertension, Epilepsy, Asthma, Ischemic heart disease

Past History:

- No history of previous hospitalization or blood transfusion
- No history of tuberculosis
- No history of pernicious anaemia

Family history:

- No history of gastrointestinal malignancy in his family.
- No history of Diabetes Mellitus, Hypertension, Tuberculosis in his family

Personal History:

- Mixed diet
- Loss of appetite present (due to abdominal pain and vomiting)
- Loss of weight present (patient was weighing 52kgs one month ago and now he weighs 45kgs. He has lost 7kgs weight in a duration of one month)
- Bowel frequency- reduced to once in 3-4days, with history of passing dark tarry stools
- Bladder habits- normal and regular
- History of alcohol consumption present (90-180ml of whisky/day since 40years) and is a chronic smoker (25 beedis/day since 40years)
- No history of any other addictive habits

Physical Examination

GENERAL SURVEY

- 70 year old male patient conscious, cooperative and well oriented to time, place and person
- Moderately built and poorly nourished(BMI-16.7)
- Temperature: 98.4 F
- BP: 100/70mmHg
- PR:96 bpm
- RR: 15/min
- SPO2: 97% at room air

- Pallor present
- No icterus
- Clubbing present- grade 2
- No cyanosis
- No pedal oedema
- No lymphadenopathy

SYSTEMIC EXAMINATION:

Per abdomen:

INSPECTION

Shape of the abdomen: flat

Umbilicus central and inverted

All corresponding quadrants move equally with respiration

No visible mass

No visible pulsations/peristalsis

No scars/sinuses

Hernial orifices- free

PALPATION

No local rise of temperaature

A vague mass is palpable in the epigastric region extending into left hypochondrium with ill defined borders, measuring 11cmx 6cm, nodular surface, hard in consistency, non-tender, moves with respiration.

The mass becomes less prominent on head raising test

It is non pulsatile

Liver and spleen: not palpable

Hernial orifices- free

PERCUSSION

Liver dullness present from left 5th intercostal space upto left costal margin

Impaired note present over the mass

Resonant note present over rest of the abdomen including the flanks

No evidence of free fluid

AUSCULTATION

Bowel sounds heard

Succussion splash: positive

Per-rectal examination:

- Inspection: normal
- Digital palpation:
    a. Tone of the sphincter : normal
    b. Within the lumen: pellets of stools present
    c. Rectal wall: Mucosa is smooth with no deposits
    d. Outside the wall: No palpable mass

Gloved finger stained with dark stools

Examination of left supraclavicular lymph nodes: not palpable

Examination of Skeletal system: No bony tenderness

Respiratory System: No features of Pleural effusion

Cardiovascular System: S1S2 present. No murmurs.

Central Nervous System: No focal neurological deficit

Summary:

- 70 year old gentleman who is a chronic smoker and alcoholic, presented with history of abdominal pain and vomiting associated with loss of appetite, loss of weight, and generalized weakness. There are no symptoms of metastasis. There is no family history of malignancy.

- Physical examination reveals anemia and a vague and hard epigastric mass that is nontender and moves with respiration with no signs of metastasis.

Provisional Diagnosis:

Ca Stomach

Reasons favoring:

- *First is his age group. Incidence of carcinoma of the stomach peaks around the age of fifty to seventy.*
- *Change in nature of pain- initially it was colicky in nature that progressed to continuous dull aching type with no pain free period.*
- *Loss of appetite and loss of weight are the cardinal symptoms of stomach cancer.*
- *He also has symptoms of upper gastrointestinal bleeding which is manifested by passing black tarry stool associated with anemia.*
- *He is also a chronic smoker and alcoholic which are also risk factors of stomach carcinoma.*
- *There is also vague mass palpable epigastric mass that is hard, nontender and moves with respiration.*

Differential Diagnoses:

1. Peptic Ulcer

Reasons favoring:

- *Firstly is his age group.*
- *Secondly, gastric ulcers usually cause loss of weight since patients are afraid to eat and the pain is associated with food intake. Epigastric pain and vomiting are present in this patient, which are symptoms favorable of a gastric ulcer. He is also anemic, that may suggest the complication of peptic ulcer which is bleeding and manifested by black tarry stool. He is also a smoker which is one of the causes of peptic ulcer disease. The patient mentioned of having a cyclic sort of pain initially, separated by a certain period of time, which is a characteristic of a gastric ulcer.*

Reasons against:

- *Duodenal ulcer patient usually have a good appetite, and taking food relieves the pain.*
- *Palpable mass*

2. Carcinoma of Pancreas

Reasons favoring:

- *Firstly is his age group which is more common at 50-70 years old.*
- *Patient also had symptoms of chronic epigastric pain and constitutional symptoms.*
- *He is a smoker which is one of the risk factors to get carcinoma of pancreas*

- *Reasons against:*
- *The pain does not associate with symptoms of obstructive jaundice, steatorrhea, diarrhea and bloating.*
- *Palpable mass that moves with respiration*

Investigations:

1.Full blood count

Parameters	
Hemoglobin	6.1
WBC	8430
Platelets	3.2
PCV	20.3
MCV	90.2
MCH	27.1
MCHC	30
Red cell count	2.25

2. Liver function test

Parameters	
Total bilirubin	1
Direct bilirubin	0.3
Indirect bilirubin	0.5
Total protein	5.6
Albumin	3.3
Globulin	2.3
AG ratio	1.48
Alkaline phosphatase	120
ALT	86
AST	94

3. Renal function test

Parameters	
Urea	5.5
Sodium	140
Potassium	4.2
Chloride	110
Creatinine	1.0

4. Coagulation profile

Parameters	
Prothrombin time	13
PT	1.1
INR	1.2
APTT	42.7
APTT ratio	1.1

5. Oesophago-Gastro-Duodeno Scopy (OGDS)

- Impression: Huge pre-pyloric tumor extending into first part of duodenum with ulcerated area. The tumor bleeds when biopsied. The pyloric ring was deformed. The esophagus and the second part of the duodenum were normal.

6. Histopathological Examination (HPE)

- Impression: Poorly differentiated adenocarcinoma

7. Electrocardiogram (ECG)

- Impression: normal ECG.

8. Chest x-ray

- Impression: there is no cannon-ball opacity noted on the lung field that may suggest presence of metastasis.

9. Computed Tomography of Thorax-Abdominal-Pelvis (CT TAP)

- Impression: pre-pyloric tumor with regional lymphadenopathy.

Final Diagnosis:

- Pre-pyloric adenocarcinoma with symptomatic anemia

Management:

1. Pre-op measures
    a. Correction of anaemia
    b. Protein supplementation
    c. Supportive measures
    d. Stomach wash using NS
    e. Arrangement of blood

2. Surgical intervention

3. Chemotherapy and radiotherapy

(FAM regime- 5FU, Adriamycin, Mitomycin C

Neoadjuvant chemotherapy- ECF- Epirubicin, cisplatin, 5FU

EOC- Epirubicin, oxaliplatin, capecitabine)

Thank You